

Anticholinergics (for Overactive Bladder)

Patient Information · Pills that calm a bladder that squeezes too soon

WARWIKI

Anticholinergics (also called antimuscarinics) are medicines that calm an **overactive bladder**. They ease sudden urges, frequent trips and urge leaks. Common ones are **oxybutynin, tolterodine, solifenacin, fesoterodine, darifenacin and trospium**. Oxybutynin also comes as a skin patch or gel. These medicines have been used for many years.

How It Works

They **block the signals** that make the bladder muscle squeeze. The bladder then squeezes less often and less suddenly as it fills, so you have less urgency and fewer trips. They help only while you take them. For most people the benefit is real but modest.

What to Expect

- Improvement comes over **a few weeks**. Your team will usually check how you are doing after 1–2 months.
- Most long-acting forms are taken **once a day**.
- **Long-acting pills, the patch and the gel** tend to cause less dry mouth than short-acting oxybutynin.
- Keep up bladder training and pelvic floor exercises for the best result.

Who Should Be Careful

- People with **narrow-angle glaucoma** (an eye condition). Tell your team about any eye problems.
- People who have **trouble emptying the bladder**, or men with a large prostate.
- People with bad constipation or some stomach and bowel conditions.
- **Older adults**, and anyone with memory problems (see page 2).

Learn the Terms

Overactive bladder (OAB)

Urgency and frequent urination from the bladder squeezing too soon.

Anticholinergic (antimuscarinic)

The group of medicines that calm the bladder muscle.

Dry mouth

The most common side effect.

Anticholinergic burden

The added effect of taking several medicines like these. It matters most in older adults.

Beta-3 agonist

A different OAB pill with less effect on memory and thinking.

Narrow-angle glaucoma

An eye condition in which these pills may be unsafe.

What about memory? Taking these medicines for a long time (more than about 3 months) has been linked to a higher risk of **dementia**. The risk seems higher with larger total doses and in older adults. Short-term use seems safe for most people. If this worries you, ask about a **beta-3 agonist** or other treatments.

How to Take It

- Take it as directed. Do not stop early just because urgency has not fully settled.
- If dry mouth bothers you, ask about a **long-acting form, the patch or the gel**.
- Tell your team about all your medicines. Some other medicines add to the same side effects.

Common Side Effects

- **Dry mouth** and **constipation** (most common)
- Blurred vision and dry eyes
- Sleepiness; in older adults, confusion
- Less sweating. Take care not to overheat in **hot weather**.

Call the office that is treating you if you have:

- A weak stream, or you feel you are not emptying
- **New confusion or memory problems**
- Bad constipation

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- You have sudden eye pain and redness with blurred vision
- You cannot reach the office and are worried

Three Things to Remember

1. These medicines calm an overactive bladder. Give them a few weeks.
2. Dry mouth and constipation are common. Long-acting forms, the patch or the gel can help.
3. Long-term use is linked to memory problems. Ask about other options if this worries you.